

WHITMORE, Harold James

DOB: March 8, 1945 | Age: 73 yrs | Sex: Male | MRN: SMC-04471932
Encounter Date: February 14, 2019 | Provider: Sandra Liu, MD — Medical Oncology | Doc ID: NCI-2019-0214-PSA

Oncology Surveillance Visit

Rising PSA — Evaluation for Biochemical Recurrence

Subjective

73-year-old, ~5 years post definitive prostate IMRT + completed ADT. Routine surveillance PSA has risen over the past year. Asymptomatic — no bone pain, weight loss, or urinary obstruction. Testosterone has recovered.

PSA Trend Since Treatment

Test	Result	Units	Reference Range	Flag
PSA — Feb 2019 (current)	1.4	ng/mL	Rising trend	H
PSA — 2018	0.7	ng/mL	Prior	
PSA — 2017	0.5	ng/mL	Prior	
PSA — 2015 (nadir)	0.2	ng/mL	Nadir	
Testosterone, total	342	ng/dL	264 - 916 (recovered)	

Assessment

Slowly rising PSA after radiotherapy. Post-radiation, biochemical failure is defined as PSA ≥ nadir + 2 ng/mL (Phoenix criteria) — current PSA 1.4 is above nadir (0.2) by 1.2, NOT yet meeting that threshold. Differential includes benign 'PSA bounce' (common 1-3 years post-RT but later here) versus early biochemical recurrence with testosterone recovery. Doubling time appears slow.

Plan

- Obtain multiparametric prostate MRI and a PSMA PET/CT to assess for local recurrence or metastatic disease (ordered today; results to follow — see imaging report 03/2019).
- Repeat PSA in 3 months to establish doubling time.
- No ADT at this time given sub-threshold, asymptomatic, slow kinetics.
- Reassure patient; multidisciplinary review pending imaging.